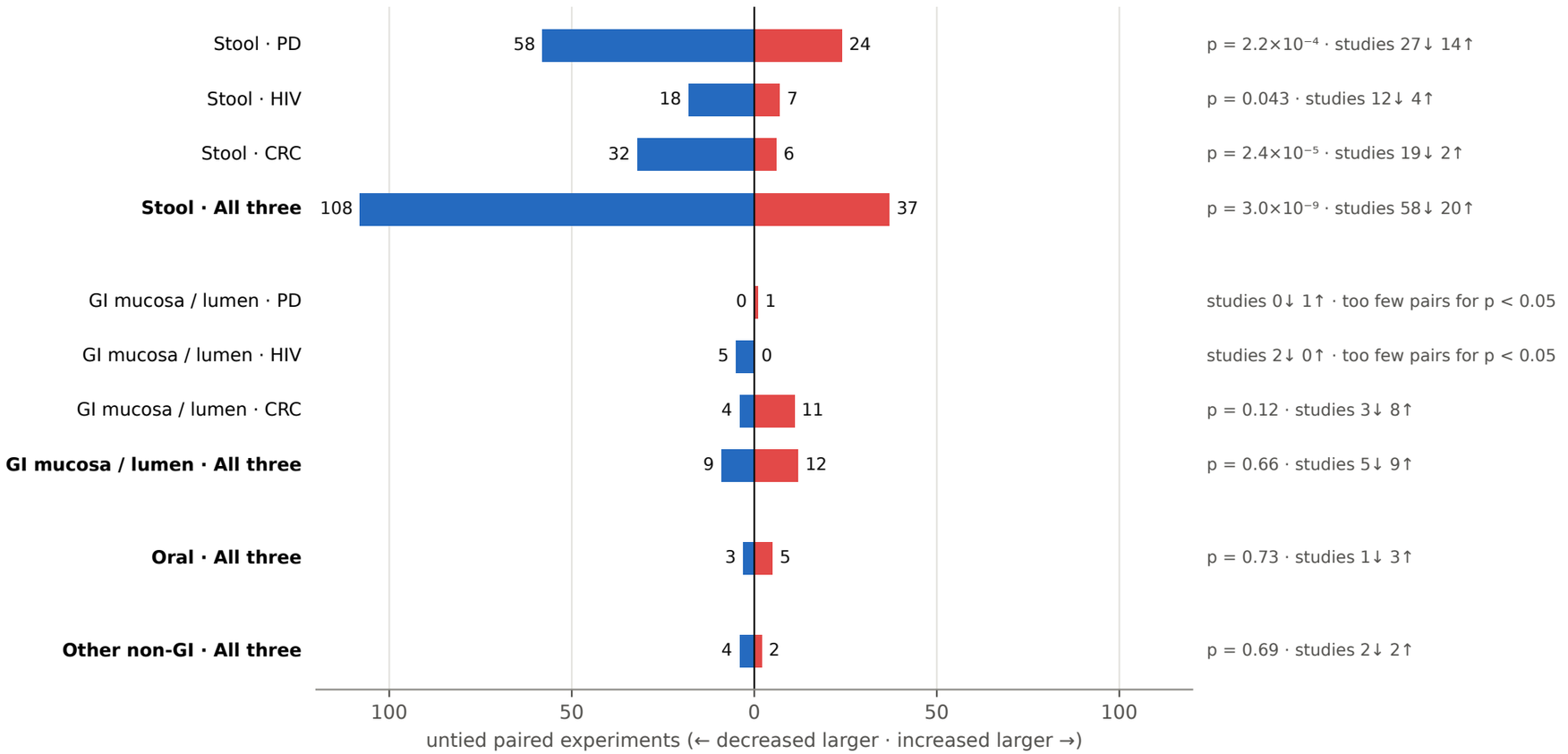
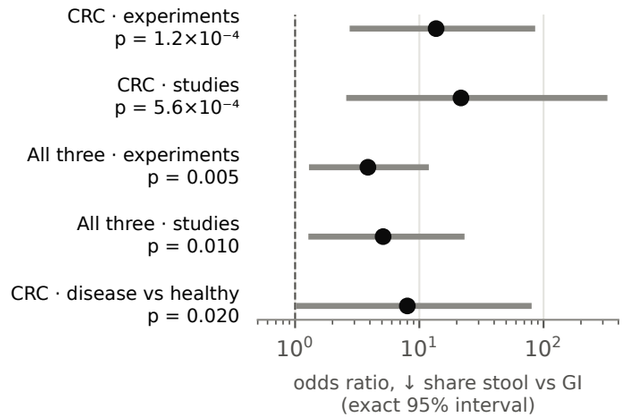
The decreased-over-increased shift is clearest in stool; other sites have too few comparisons to judge

Across PD, HIV and CRC stool, 108 of 145 untied paired experiments had the larger cellulose annotation share in the decreased signature ($p = 3.0\times10^{-9}$); outside stool most strata have fewer than six untied pairs.

A. Paired experiments: decreased signature larger (↓) or smaller (↑)



B. Stool vs GI mucosa / lumen



C. Decreased-signature studies by site

	PD	HIV	CRC
Stool	49	19	31
GI mucosa / lumen	2	4	14
Oral	2	3	4
Other non-GI	3	3	0

Paired experiment: one BugSigDB experiment with both an increased and a decreased signature; the share compared is cellulose annotations over all retained non-MediaDive annotations. Ties excluded. Study rows take the sign of each study's mean paired difference. The Fisher tests treat units as independent; a few studies sampled both compartments, and the sites differ in sampling effort (C), so a difference between compartments is not evidence of anatomical specificity. PD, Parkinson's disease; HIV, HIV infection; CRC, colorectal cancer. BugSigDB 2026-04-24_00:41_UTC; human profiles; condition and body site from each experiment's own record.